# ATS Triage for Urgent & Primary Care

## *Education Workbook*

*Shell Piercy*



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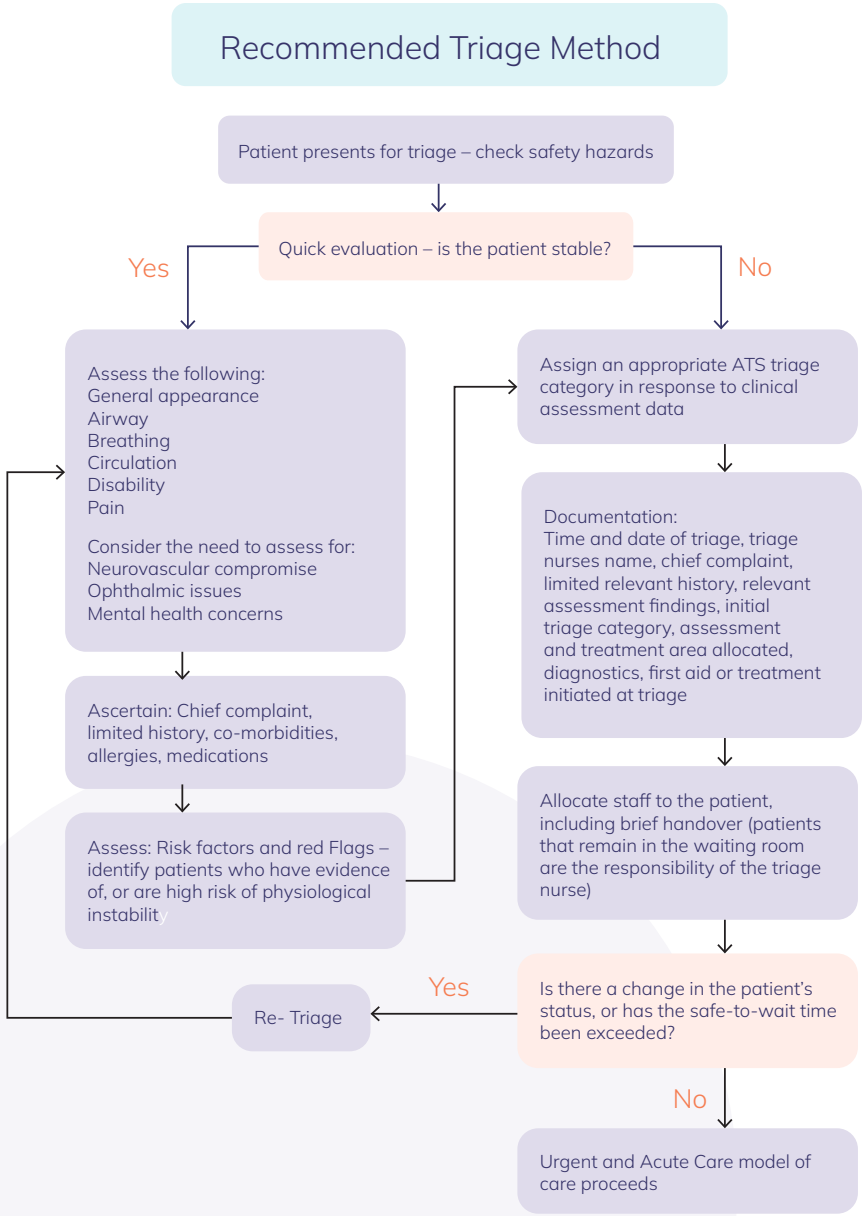
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# Triage Process Flowchart



## TRIAGE ASSESSMENT FORM

Nurse:_____ #:_____ Signature:_____ Patient:_____ NHI:_____ DOB:_____

Time:_____ Date:_____ Allergies/Alerts:_____

Medications:_____

Chief Complaint/Presenting Problem:_____

Limited Relevant History:_____

General Appearance:_____

| Triage Code | ATS 1 | ATS 2 | ATS 3 | ATS 4 | ATS 5 |
|---|---|---|---|---|---|
| Airway | ☐ Immediate Risk<br>☐ Obstructed | ☐ Potential risk<br>☐ Partially obstructed<br>☐ Anaphylaxis<br>☐ Drooling with distress<br>☐ Severe stridor | ☐ Potent no risk | ☐ Potent no risk | ☐ Potent no risk |
| Breathing | ☐ Respiratory arrest<br>☐ Severe respiratory Distress<br>☐ Hypoventilation RR <10<br>☐ Central Cyanosis<br>☐ Severe or no use of accessory muscles | ☐ Moderate respiratory distress<br>☐ Peripheral Cyanosis<br>☐ Speaking only words<br>☐ Moderate use of accessory muscles | ☐ Mild respiratory distress<br>☐ Speaking in short 3-4 words per breath<br>☐ Minimal use of accessory muscles | ☐ No respiratory distress. No SOB. No WOB.<br>☐ Speaking full sentences | ☐ No respiratory distress. No SOB. No WOB.<br>☐ Speaking full sentences |
| Circulation | ☐ Cardiac arrest<br>☐ Severe haemodynamic compromise<br>☐ Symptomatic BP <80mmHg or no radial pulse<br>☐ Uncontrolled haemorrhage<br>☐ Pale, diaphoretic cold skin | ☐ Moderate haemodynamic compromise<br>☐ Non symptomatic BP <80mmHg<br>☐ Abnormal pulse rate <50 or >150<br>☐ Absent radial pulse<br>☐ Pale, diaphoretic cold skin | ☐ Mild haemodynamic compromise<br>☐ Pale, cool skin<br>☐ Weak radial pulse | ☐ No haemodynamic compromise<br>☐ Warm, dry and well perfused skin<br>☐ Normal pulse characteristics at radius | ☐ No haemodynamic compromise<br>☐ Warm, dry and well perfused skin<br>☐ Normal pulse characteristics at radius |
| Disability | ☐ Unresponsive or responds to pain<br>☐ GCS <9<br>☐ Ongoing Seizure | ☐ Responds to pain<br>☐ GCS 9-12 | ☐ Responds to pain or voice<br>☐ GCS >12 | ☐ Alert<br>☐ GCS 15 | ☐ Alert<br>☐ GCS 15 |
| Pain | | ☐ Severe pain 7/10-10/10<br>☐ Abbey Scale 14+ Severe | ☐ Moderate Pain 4/10-6/10<br>☐ Abbey Scale 8-13 Moderate | ☐ Mild Pain 1/10-3/10<br>☐ Abbey Scale 3-7 Mild | ☐ No Pain<br>☐ Abbey Scale 0-2 No pain<br>☐ Minimal Pain |
| Neurovascular Compromise | | ☐ Severe, cold<br>☐ Pulseless<br>☐ Dislocations<br>☐ Altered Sensation | ☐ Moderate, cool<br>☐ Pulses weak<br>☐ Altered Sensation<br>☐ Delayed cap refil peripherally | ☐ No compromise<br>☐ Colour, warmth<br>☐ Sensation and movement normal | ☐ No compromise<br>☐ Colour, warmth<br>☐ Sensation and movement normal |
| Opthalmic | | ☐ Penetrating Eye injury<br>☐ Chemical injury<br>☐ Sudden loss of vision with or without injury<br>☐ Sudden onset of severe eye pain, vision change and redness<br>☐ Orbital Cellulitis extending to forehead<br>☐ Globe rupture | ☐ Sudden abnormal vision with or without injury<br>☐ Moderate eye pain -blunt eye injury -flash burns -foreign body<br>☐ Hyphemia<br>☐ Detached retina<br>☐ Orbital Cellulitis | ☐ Normal Vision<br>☐ Mild eye pain -blunt eye injury -Flash burns -Foreign body | ☐ Normal vision<br>☐ No eye pain<br>☐ Conjuctivitis<br>☐ Subconjunctival bleeding<br>☐ Sticky or red itchy eye |
| Mental Health | ☐ Violence to self or others<br>☐ Threat of violence to self or others with means or weapon | ☐ Risk of harm to self or others<br>☐ Severe behavioral disturbance | ☐ Possible harm to self or others<br>☐ Moderate behavioural disturbance<br>☐ Severe distress | ☐ No immediate risk to self or others<br>☐ Semi urgent mental health problem<br>☐ Medication compliance request | ☐ No immediate risk to self or others<br>☐ Known patient with chronic symptoms<br>☐ No acute distress or behavioural disturbance |

| Risk Factors | ☐ Mechanism of injury | ☐ Comorbidities and age | ☐ Time critical treatment | ☐ Events preceding presentaion | ☐ Red alert pesentation | ☐ High risk patient | ☐ Pregnancy | ☐ Pain priority |
|---|---|---|---|---|---|---|---|---|

Risk Factor:_____

Triage Vitals: Pulse Rate:_____ Pulse Characteristics:_____ Respiratory Rate:_____ Spoz:_____

Triage Assessment:_____



# Quick Reference Guide

## ATS categories for treatment acuity and performance thresholds

| ATS Category | Treatment Acuity (maximum waiting time for assessment and treatment by RN or/and NP/DR) | Performance Indicator |
|---|---|---|
| 1 | Immediate | 100% of the time |
| 2 | 10 minutes | 80% of the time |
| 3 | 30 minutes | 75% of the time |
| 4 | 60 minutes | 70% of the time |
| 5 | 120 minutes | 70% of the time |

# Adult Triage Tool

| Criterion | Category 1<br>Immediate | Category 2<br>10 minutes | Category 3<br>30 minutes | Category 4<br>60 minutes | Category 5<br>120 minutes |
|---|---|---|---|---|---|
| **Airway**<br>Risk ATS 1<br>Potential Risk ATS 2<br>No Risk ATS 3-5 | – Obstructed<br>– Immediate airway risk | – Partially obstructed<br>– Anaphylaxis<br>– Drooling w distress<br>– Severe stridor<br>– Potential airway risk | – Patent<br>– No airway risk | – Patent<br>– No airway risk | – Patent<br>– No airway risk |
| **Breathing**<br>Severe Resp Distress ATS 1<br>Moderate Resp Distress ATS 2<br>Mild Resp Distress ATS 3<br>No Resp Distress ATS 4-5 | – Severe respiratory distress/absent respiration/ hypoventilation<br>– RR <10<br>– Central cyanosis<br>– Either severe or no use of accessory muscles | – Moderate respiratory distress<br>– Peripheral cyanosis<br>– Speaking in words<br>– Moderate use of accessory muscles | – Mild respiratory distress<br>– Speaking in short sentences 3-4 words per breath<br>– Minimal use of accessory muscles | – No respiratory distress<br>– No SOB<br>– No WOB<br>– Speaking full sentences | – No respiratory distress<br>– No SOB<br>– No WOB<br>– Speaking full sentences |
| **Circulation**<br>Severe Shock ATS 1<br>Moderate Shock ATS 2<br>Mild Shock ATS 3<br>No Shock ATS 4-5 | – Severe haemodynamic compromise/ absent circulation<br>– Uncontrolled haemorrhage<br>– Cardiac arrest<br>– Symptomatic<br>– BP < 80mmhg (no radial pulse) | – Moderate haemodynamic compromise<br>– Non symptomatic<br>– BP <80mmgh (no radial pulse) | – Mild haemodynamic compromise | – No haemodynamic compromise | – No haemodynamic compromise |
| **Circulation**<br>GCS or AVPU | – GCS <9<br>– Unresponsive or responds to pain | – GCS 9–12<br>– Responds to pain | – GCS >12<br>– Responds to pain or voice | – Normal GCS 15<br>– Alert | – Normal GCS 15<br>– Alert |
| **Pain**<br>Objective and Subjective Assessment | | – Severe Pain<br>– 7/10 - 10/10 | – Moderate Pain<br>– 4/10 – 6/10 | – Mild Pain<br>– 1/10 – 3/10 | – No Pain |
| **Neurovascular**<br>Colour, Warmth, Movement and Sensation | | – Severe neurovascular compromise<br>– Pulseless, cold, altered sensation, delayed peripheral cap refill<br>– Dislocations | – Moderate neurovascular compromise<br>– Weak pulse present, cool, altered sensation, delayed peripheral cap refill | – Mild neurovascular compromise<br>– Normal pulse, warm, mild altered sensation, normal peripheral cap refill | – No neurovascular compromise |

## Adult Vital Signs

| | Heart Rate | Resp Rate | SBP | Cap Refill | Temp | Sp02 |
|---|---|---|---|---|---|---|
| Normal Adult | 60 - 100 bpm | 12 - 20 bpm | 90 - 120 | < 2 sec | 36.5 - 37.5c | > 94% ra |
| COPD Adult | 60 - 100 bpm | Increased RR | 90 - 120 | < 2 sec | 36.5 - 37.5c | 88 - 92% ra |

Reference : National EWS Chart



## Levels of Compromise: Circulation

| Severe Haemodynamic Compromise<br>Severe Shock<br>ATS 1 | Moderate Haemodynamic Compromise<br>Moderate Shock<br>ATS 2 | Mild Haemodynamic Compromise<br>Mild Shock<br>ATS 3 | No Haemodynamic Compromise<br>No Shock<br>ATS 4-5 |
|---|---|---|---|
| The patient is severely compromised if their HR, RR, Cap Refill, GCS and Pulse are markedly abnormal such that there is a high risk of cardiac arrest, unless immediate interventions are carried out.<br>Severe hypotension, or absent radial pulses or an inability to obey commands, or severe shortness of breath.<br>The assessment of the level of cardiovascular compromise can be further validated by a visual assessment of the patient. 'looks ATS 1' the patient is usually severely compromised.<br>Pale, diaphoretic and cold with grossly abnormal to RR and levels of GCS | The patient is moderately compromised if their RR, GCS/ AVPU, HR, Pulse and Cap Refill are abnormal or significant symptoms of MI are present. For example, when the patient has: hypotension or prolonged capillary refill time, or an altered level of consciousness with an ability to obey commands, or moderate shortness of breath.<br>The assessment of the level of cardiovascular compromise can be further validated by a visual assessment of the patient. 'looks ATS 2' the patient is usually moderately compromised.<br>Pale, diaphoretic, cold. | Mildly compromised if their RR, Pulse, Peripheral Cap Refill, normal GCS / AVPU, are near to normal and / or mild symptoms of MI are present.<br>The assessment of the level of cardiovascular compromise can be further validated by a visual assessment of patient. 'looks ATS 3' the patient is usually mildly compromised.<br>May be pale and cool. | Not compromised if their HR, RR, Pulse and Peripheral Cap Refill are normal and there are no symptoms of MI present.<br>The assessment of the level of cardiovascular compromise can be further validated by a visual assessment of the patient. 'looks ATS 4 - 5' the patient is usually not compromised.<br>Warm, dry and well perfused. |

## Levels of Compromise: Breathing

| Severe Respiratory Distress / Absent Respiration<br>ATS 1 | Moderate Respiratory Distress<br>ATS 2 | Mild Respiratory Distress<br>ATS 3 | No Respiratory Distress<br>ATS 4 - 5 |
|---|---|---|---|
| – Extremely short of breath, or hypoventilation < 10bpm<br>– Unable to speak<br>– May or may not have adventitious lung sounds<br>– Severe or absent use of accessory muscles<br>– Severe agitation or decreased LOC<br>– Rapidly falling Sp02<br>– (Hypoventilation with absent adventitious lung sounds and no use of accessory muscles is a very late sign – imminent respiratory arrest)<br>– Central and peripheral cyanosis | – Very short of breath<br>– Peripheral cyanosis<br>– Speaking in only words per breath<br>– May only have quiet adventitious lung sounds<br>– Significant chest / neck indrawing<br>– Tripod position<br>– May be agitated<br>– Sp02 < 90% ra<br>– Moderate use of accessory muscles | – Mild respiratory distress<br>– Speaking in short sentences 3-4 words per breath<br>– Minimal use of accessory muscles<br>– May have loud adventitious lung sounds<br>– Normal LOC<br>– Normal Sp02 for patient | – No SOB, No WOB |

# The ABCs of Obstetrics

| Urgency | Urgency Indicator | Reason for Caution |
|---|---|---|
| Airway | - Any potential compromise<br>- Increased nasal congestion | Often difficult intubations due to:<br>• Increased patient size<br>• Difficulty with patient positioning<br>• Different induction agents required<br>- Increased vascularity of nose and airways causes difficulty in breathing |
| Breathing | - Asthma | - Progesterone may be responsible for increased drive to breathe<br>- One third of pregnant asthmatic women experience a deterioration in their condition |
| Circulation | - Palpations<br>- Headaches<br>- Sudden drop in BP<br>- Symptoms of pulmonary embolus | - Progesterone causes widespread vasodilatation<br>- Oestrogen may contribute to increases in blood volume<br>- Diastolic BP - 16-17mmHg decrease<br>- BP lowest during the second trimester<br>- Cardiac output (CO) – by 30-50% increase<br>- Hyperdynamic flow<br>- High volume and dynamic flow may cause cerebral hemorrhage, especially in subarachnoid hemorrhage (SAH) during pregnancy<br>- Sudden and serious deterioration of their condition<br>- Changes in coagulation system associated with pregnancy |

## Points to Remember

- Hyperdynamic and physiological changes occur as early as 6-8 weeks gestation.
- An assessment of urgency must be made on the basis of both the woman and the foetus.
- An elevated BP is an ominous sign: the higher the BP the more urgent the review.
- At 20 weeks the weight of the uterus compresses the inferior vena cava if the woman is lying on her back - a compromise to foetal wellbeing.
- The risk of many conditions is higher in pregnant women than non-pregnant women of childbearing age.
- These conditions include:

- ceberal hemorrhage or cerebral thrombosis
- severe pneumonia
- atrial arrythmias
- venous thrombosis
- chlolelithiasis
- pyelonephritis
- spontaneous arterial dissections
  e.g. splenic and subclavian dissections, with no previous medical history.

- Domestic violence is more prevalent during pregnancy. This can mean increased complications for mother and adverse neonatal outcomes.
- In the setting of trauma, maternal signs may remain stable even when loss of one-third of blood volume may have occurred.
- The best initial treatment for the foetus is the optimum resuscitation of the mother.



# Normal Vital Signs in Pregnancy

| Vital Sign | First Trimester | Second Trimester | Third Trimester |
|---|---|---|---|
| Systolic Blood Pressure mmHg | 94.8 to 137.6 | 95.6 to 136.4 | 101.6 to 143.5 |
| Diastolic Blood Pressure mmHg | 55.5 to 86.9 | 56.8 to 87.1 | 62.4 to 94.7 |
| Heart Rate Beats Per Minute | 63.1 to 105.2 | 67.4 to 112.5 | 64.5 to 113.8 |
| Respiratory Rate | 8 to 24 | 8 to 24 | 8 to 24 |
| Oxygen Saturation SpO2 (%) | 94.3 to 99.4 | 92.9 to 99.3 | 93.4 to 98.5 |
| Temperature | 35.55 to 37.51 | 35.35 to 37.37 | 35.37 to 37.35 |

Reference : Green LJ, et. al,Gestation-Specific Vital Sign Reference Ranges in Pregnancy. Obstet Gynecol. 2020 Mar;135(3):653-664. PMID: 32028507

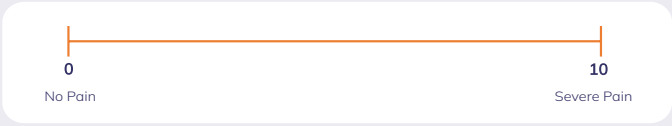
## Pain Assessment: Numerical Scale 0-10

Validated Methods for Quantitative Assessment of Pain

Visual analogue scale: 100mm line

Nelson, Cohen, Lander, et al 2004

Use 100mm line as shown




Ask the patient to mark their level of pain on the line.

# Abbey Pain Scale

For measurement of pain in people who cannot verbalise

How to use scale: While observing the patients, score questions 1 to 6

**Q1. Vocalisation**
eg: whimpering, groaning, crying

Absent 0 Mild 1 Moderate 2 Severe 3

**Q2. Facial Expression**
eg: looking tense, frowning, grimacing, looking frightened

Absent 0 Mild 1 Moderate 2 Severe 3

**Q3. Change in Body Languange**
eg: fidgeting, rocking, guarding part of body, withdrawn

Absent 0 Mild 1 Moderate 2 Severe 3

**Q4. Behavioural Change**
eg: increased confusion, refusing to eat, alteration in usual patterns

Absent 0 Mild 1 Moderate 2 Severe 3

**Q5. Physiological Change**
eg: temperature, pulse or blood pressure outside normal limits, perspiring, flushing or pallor

Absent 0 Mild 1 Moderate 2 Severe 3

**Q6. Physical Changes**
eg: skin tears, pressure areas, arthritis, contractures, previous injuries

Absent 0 Mild 1 Moderate 2 Severe 3

Add scores 1-6 and record here.
Total Pain Score =

Now tick the box that matches the Total Pain Score:

| 0-2 No Pain | 3-7 Mild | 8-13 Moderate | 14+ Severe |
|---|---|---|---|

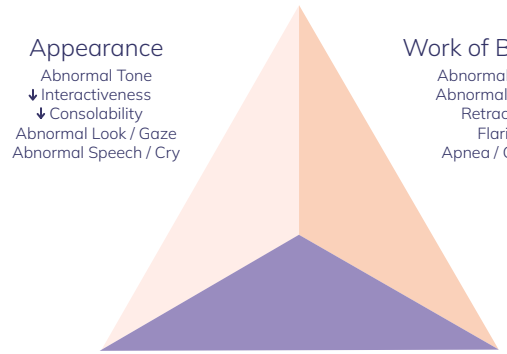
Finally, tick the box that matches the type of pain:

| Chronic | Acute | Acute on chronic |
|---|---|---|



# Paediatric Triage

## PAEDIATRIC ASSESSMENT TRIANGLE



**Appearance**
Abnormal Tone
↓ Interactiveness
↓ Consolability
Abnormal Look / Gaze
Abnormal Speech / Cry

**Work of Breathing**
Abnormal Sounds
Abnormal Position
Retractions
Flaring
Apnea / Gasping

**Circulation to Skin**
Pallor
Mottling
Cyanosis

## Normal Paediatric Vital Sign Ranges: By Age Group

| Age | Heart Rate | Respiratory Rate | SBP | Cap Refill | Temperature | Sp02 |
|---|---|---|---|---|---|---|
| < 3 months | 110 - 150 bpm | 30 - 60 bpm | 60 - 80 mmHg | < 2 sec | 36.5 - 37.5c | > 94% ra |
| 3 - 12 months | 100 - 150 bpm | 25 - 50 bpm | 80 - 100 mmHg | < 2 sec | 36.5 - 37.5c | > 94% ra |
| 1 - 5 years | 90 - 120 bpm | 20 - 40 bpm | 90 - 110 mmHg | < 2 sec | 36.5 - 37.5c | > 94% ra |
| 5 -12 years | 70 - 110 bpm | 20 - 30 bpm | 90 - 120 mmHg | < 2 sec | 36.5 - 37.5c | > 94% ra |
| > 12 years | 60 - 100 bpm | 12 - 17 bpm | 100 - 130 mmHg | < 2 sec | 36.5 - 37.5c | > 94% ra |

Reference : CMDHB, Kids First, Bedside PEWS Score

# Paediatric Triage Tool

| | ATS 1 | ATS 2 | ATS 3 | ATS 4 | ATS 5 |
|---|---|---|---|---|---|
| **Airway**<br>Red Flags = unable to verbalise, no cry, drooling, difficulty swallowing, stridor! | – Obstructed<br>– Partially obstructed with severe respiratory distress<br>– Severe stridor<br>– Drooling | – Patent<br>– Partially obstructed with moderate respiratory distress<br>– Biphasic Stridor Minor drooling Severe Croup | – Patent<br>– Partially obstructed with mild respiratory distress<br>– Croup | – Patent | – Patent |
| **Breathing**<br>WOB = nasal flaring, grunting, subclavian retractions, tracheal tug, intercostal indrawing, abdo breathing, head bobbing<br><br>Red Flags = grunting, head bobbing, pallor/ cyanosis, agitated / inconsolable, floppy / lethargic!<br>pre-terminal = floppy, cyanosed, bradycardic! | – Absent respiration or hypoventilation<br>– Severe respiratory distress, e.g.<br>– Severe use accessory muscles<br>– Severe retraction<br>– Acute cyanosis<br>– Not speaking<br><br>Extreme / Absent WOB | – Respiration present<br>– Moderate respiratory distress, eg.<br>– Moderate use accessory muscles<br>– Moderate retraction<br>– Skin pale<br>– Speaking single words per breath<br><br>Severe WOB | – Respiration present<br>– Mild respiratory distress, e.g.<br>– Mild use accessory muscles<br>– Mild retraction<br>– Skin pink.<br>– Speaking short sentences<br><br>Moderate WOB | – Respiration present<br>– No respiratory distress<br>– Mild use accessory muscles<br>– No retraction, no abdo breathing, no nasal flaring.<br>– Speaking full sentences<br><br>Mild / No WOB | – Respiration present<br>– No respiratory distress<br>– No use accessory muscles<br>– No retraction.<br>– Speaking full sentences<br><br>No SOB / No WOB |
| **Circulation**<br>Assess peads for - tachy or brady, skin colour, cap refill, bleeding, dehydration (wet diapers, feeding, dry mucosa) | – Absent circulation or significant bradycardia e.g. HR <60 in an infant (that looks unwell)<br>– Severe haemodynamic compromise, e.g.<br>– absent peripheral pulses<br>– skin pale<br>– cold, moist, mottled<br>– significant tachycardia capillary refill >4 secs.<br><br>Uncontrolled hemorrhage | – Circulation present<br>– Moderate hemodynamic compromise, e.g.<br>– weak/ threadybrachial pulse<br>– skin pale, cool,<br>– moderate tachycardia capillary refill 3 – 4 secs.<br>– Severe signs of dehydration<br><br>> 6 s/s dehydration | – Circulation present<br>– Mild haemodynamic compromise, e.g.<br>– palpable peripheral pulse,<br>– skin pale, warm<br>– mild tachycardia. cap refill < 2 secs<br>– Moderate signs of dehydration<br>– Less wet diapers, Hx of gastro illness, dry mucosa, sunken fontanelle<br><br>3–6 s/s dehydration | – Circulation present<br>– No haemodynamic compromise, e.g.<br>– palpable peripheral pulses<br>– skin pink, warm, dry.<br>– Mild signs of dehydration normal cap refill<br><br>< 3 s/s dehydration | – Circulation present<br>– No haemodynamic compromise, e.g.<br>– palpable peripheral pulses<br>– Skin pink, warm, dry.<br>– Normal cap refill<br><br>No s/s dehydration |

| | ATS 1 | ATS 2 | ATS 3 | ATS 4 | ATS 5 |
|---|---|---|---|---|---|
| **Disability**<br>AVPU | GCS < 8<br>– Ongoing seizure<br>– Flacid<br>– Tonic – clonic<br>– Tonic only seizure.<br>– Responds to pain or unconscious | GCS 9–12<br>– Severe decrease in activity, e.g.<br>– no eye contact,<br>– decreased muscle tone<br>– responds to voice or pain | GCS > 13<br>– Moderate decrease in activity, e.g.<br>– lethargic<br>– eye contact when disturbed.<br>– responds to voice | Normal GCS (15) or no acute change to usual GCS.<br>– Mild decrease in activity, e.g.<br>– quiet but eye contact<br>– interacts with parents.<br>– alert | Normal GCS (15) or no acute change to usual GCS.<br>– No alteration to activity, e.g.<br>– playing<br>– smiling.<br>– alert |
| **Pain Assessment**<br>Wong Baker Faces<br>FLACC | | Severe Pain 8-10/10<br>e.g.<br>– patient/ parents report severe pain<br>– skin, pale, cool<br>– alteration in vital signs<br>– requests analgesia. | Moderate Pain<br>e.g.<br>– patient/ parents report moderate pain<br>– skin, pale, warm<br>– alteration in vital signs<br>– requests analgesia. | Mild Pain<br>e.g.<br>– patient/parents report mild pain<br>– skin, pink, warm<br>– no alteration in vital signs<br>– requests analgesia. | No or Mild Pain<br>e.g.<br>– patient/ parents report mild pain<br>– skin, pink, warm<br>– no alteration in vital signs<br>– declines analgesia. |
| **Neurovascular Assessment**<br>Use play to assess status | | Severe Neurovascular Compromise<br>e.g.<br>– pulseless<br>– cold<br>– nil sensation<br>– nil movement<br>– capillary refill. | Moderate Neurovascular Compromise<br>e.g.<br>– pulse present<br>– cool<br>– sensation<br>– movement<br>– capillary refill. | Mild Neurovascular Compromise<br>e.g.<br>– pulse present<br>– normal / sensation<br>– normal / movement<br>– normal capillary refill. | No Neurovascular Compromise |

# Assessment of Dehydration Levels in Infants

| SIGNS | SEVERITY | | |
|---|---|---|---|
| | Mild | Moderate | Severe |
| General Condition | Thirsty, restless, irritable | Thirsty, restless, agitated | Withdrawn, somnolent or comatose; rapid deep breathing |
| Pulse | Normal | Rapid, weak | Rapid, weak |
| Anterior Fontanelle | Normal | Sunken | Very sunken |
| Eyes | Normal | Sunken | Very sunken |
| Tears | Present | Absent | Absent |
| Mucous Membranes | Slightly dry | Dry | Dry |
| Skin Turgor | Normal | Decreased | Decreased with tenting |
| Urine | Normal | Reduced, concentrated | None for several hours |
| Weight Loss | 4-5% | 6-9% | >10% |



# FLACC Behavioural Pain Scale

| | 0 | 1 | 2 |
|---|---|---|---|
| Face | No particular expression or smile | Occasional grimace or frown, withdrawn, disinterested | Frequent to constant quivering chin, clenched jaw |
| Legs | Normal position or relaxed | Uneasy, restless, tense | Kicking or legs drawn up |
| Activity | Lying quietly, normal position, moves easily | Squirming, shifting back and forth, tense | Arched, rigid or jerking |
| Cry | No cry (awake or sleep) | Moans or whimpers; occasional complaint | Crying steadily, screams, sobs, frequent complaints |
| Consolability | Content, relaxed | Reassured by touching, hugging or being talked to, distractible | Difficult to console or comfort |

## Instructions:

Patients who are awake:
- Observe for at least 2-5 minutes
- Observe legs and body uncovered
- Reposition patient or observe activity, assess body for tenseness and tone
- Initiate consoling interventions if needed

Patients who are asleep:
- Observe for at least minutes or longer
- Observe body and legs uncovered.
- If possible reposition the patient.
- Touch the body and assess for tenseness and tone.

Each category is scored on a 0-2 scale which results in a total score of 0-10

Assessment of Behavioural Score:
0 = Relaxed and comfortable
1 - 3 = Mild discomfort
4 - 6 = Moderate pain
7 - 10 = severe discomfort/pain

Reference: Merkel S, Voepel-Lewis T, Shayevitz JR, et al: The FLACC: A behavioural scale for scoring postoperative pain in young children. Pediatric Nursing 1997;23:293-797

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# Wong-Baker FACES Pain Rating Scale

Developed for young patients to communicate how much pain they are feeling.




INSTRUCTIONS

Explain to the child that each face is for a person who feels happy because he has no pain (hurt) or sad because he has some or a lot of pain.

Face 0 is very happy because he doesn't hurt at all.
Face 2 hurts just a little bit.
Face 4 hurts a little more.
Face 6 hurts even more.
Face 8 hurts a whole lot more.
Face 10 hurts as much as you can imagine, although you do not have to be crying to feel this bad.

Ask the child to choose the face that best describes how he/she is feeling.

Reference: Hockenberry MJ, Wilson D, Winkestein ML: *Wong's Essentials of Pediatric Nursing.* ed 7, St. Louis, 2005, p. 1259. Used with permission. Copyright, Mosby.
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# Risk Factors

consideration for triage Adults and Children

Paedriatic specific risks in RED

| Age | Co-morbidities | Events Preceding Presentation | Mechanism of Injury (MOI) | Red Flags / Cautions |
|---|---|---|---|---|
| - ≤ to 65<br>- Poly pharmacy<br>- Comorbidities<br>- ≥ to 28 days Stable Neonate ATS 3 Unstable Neonate ATS 2 | - Hx prematurity graduate of Neonates<br>- Respiratory disease<br>- Cardiovascular disease<br>- Renal disease<br>- Cancer<br>- Diabetes<br>- Substance abuse<br>- Immuno-compromised<br>- Congenital disease<br>- Complex medical Hx<br>- Intellectual disability<br>- Cognitive impairment<br>- English second language<br>- Hearing aids/sight impaired<br>- Transplant patients<br>- Polypharmacy<br>- Chicken pox – Reyes Syndrome (Aspirin and Ibuprofen) | - Apnoeic/cyanotic episode<br>- Seizure activity<br>- Decreased intake decreased output<br>- Red current jelly stool<br>- Bile stained vomiting<br>- Parental concern<br>- Paediatric overdose<br>- Fever in a child less then one year<br>- Collapse vs fall<br>- Recent illness<br>- Fever in Adults<br>- Hx of infectious contact | - Penetrating injury<br>- Fall >2 x height<br>- RTC/MVA >60 kph<br>- MBA/cyclist > 30 kph<br>- Pedestrian<br>- Ejection/rollover<br>- Prolonged extrication (>30 minutes)<br>- Death in same car<br>- Staring on windscreen<br>- Compound or major #<br>- Dislocation of large joint<br>- Explosion<br>- Prolonged crush<br>- Severe localised trauma, #, dislocation, amputation, lac | - Thunder clap headache – subarachnoid bleed<br>- Elderly patients on Anticoagulants or Betablockers<br>- Situational Urgency (Abuse)<br>- AAA – ABDO/BACK Pain<br>- Torsion testicle/ ovary<br>- Meningitis<br>- Atypical MI<br>- Significant sedative or intoxication, drugs/ ETOH<br>- Acute stoke<br>- Sepsis / Neuropenic sepsis<br>- NAI children or adults<br>- Paradoxical behaviours<br>- Drooling<br>- Toxic looking child<br>- Significant dehydration<br>- Pregnancy 3rd trimester pregnant women with headache or dizziness. Pre-Eclampsia. Ectopic. Shock may be masked. Gestation > 24 wks is considered 2 Pt's |

# Opthalmology Triage Tool

| Category 1<br>Immediate | Category 2<br>10 minutes | Category 3<br>30 minutes | Category 4<br>60 minute | Category 5<br>120 minutes |
|---|---|---|---|---|
| | - Penetrating eye injury<br>- Chemical injury<br>- Sudden loss of vision with or without injury<br>- Sudden onset severe eye pain, vision change and redness<br>- Orbital cellulitis extending to forehead<br>- Globe rupture – tear drop pupil | - Sudden abnormal vision with or without injury<br>- Moderate eye pain, e.g.<br>• blunt eye injury<br>• flash burns<br>• foreign body<br>- Hyphemia – bleeding into cornea & iris space<br>- Detached retina – floaters, loss of vision in peripheral fields.<br>- Globe rupture – tear drop pupil.<br>- Orbital cellulitis | - Normal vision<br>- Mild eye pain, e.g.<br>• blunt eye injury<br>• flash burns<br>• foreign body | - Normal vision<br>- No eye pain<br>- Conjunctivitis<br>- Bleeding in eye<br>- Sticky or red itchy eye |



# Mental Health Triage Tool

| Triage Code – Treatment Acuity | Description | Typical Presentation | General Management Principles * |
|---|---|---|---|
| 1 – Immediate<br>Right Now<br>(You would definitely cross the street if you saw this person) | Definite danger to life (self or others)<br>Australasian Triage Scale states:<br>- Severe behavioural disorder with immediate threat of dangerous violence (breaking laws) | Observed - as seen at triage<br>- Violent behaviour<br>- Possession of weapon<br>- Self-destruction in Clinic<br>- Extreme agitation or restlessness<br>- Bizarre/disoriented behaviour<br>Reported<br>- Verbal commands to do harm to self or others, that the person is unable to resist (command hallucinations)<br>- Recent violent behaviour | Supervision<br>Continuous visual surveillance 1:1 ratio (see definition below)<br>Action<br>- Alert clinical medical staff immediately<br>- Alert mental health triage or equivalent<br>- Provide safe environment for patient and others<br>- Ensure adequate personnel to provide restraint/detention based on industry standards<br>You will<br>- Call security +/- police if staff or patient safety compromised. May require several staff to contain patient<br>- 1:1 observation<br>- Intoxication by drugs and alcohol may cause an escalation in behaviour that requires management. |

Mental illness is not often acute! Consider medical causes!

UTI/infections, head injury / TBI / bleed / stroke, hypoxia, hypoglycemia, hypothermia, ETOH / withdrawal, CNS infection, metabolic, renal failure, space occupying lesions

# Mental Health Triage Tool

| Triage Code – Treatment Acuity | Description | Typical Presentation | General Management Principles* |
|---|---|---|---|
| 2 – Emergency<br>Within 10 minutes<br>(You would probably cross the street if you saw this person) | Probable risk of danger to self or others<br>*AND/OR*<br>Severe behavioural disturbance<br>Australasian Triage Scale states:<br>Violent or aggressive (if):<br>- Immediate threat to self or others<br>- Requires or has required restraint<br>- Severe agitation or aggression | Observed<br>- Extreme agitation/ restlessness<br>- Physically/verbally aggressive<br>- Confused/unable to cooperate<br>- Hallucinations/ delusions/paranoia<br>- Requires restraint/ containment<br>- High risk of absconding and not waiting for treatment – Risk to self and others!<br>Reported<br>- Attempt at selfharm/ threat of self-harm<br>- Threat of harm to others<br>- Unable to wait safely<br>- Self harm with plan to continue self harm | Supervision<br>Continuous visual supervision (see definition below)<br>Action<br>- Alert clinical medical staff immediately<br>- Alert mental health triage<br>- Provide safe environment for patient and others<br>- Use defusing techniques (oral medication, time in quieter area)<br>- Ensure adequate personnel to provide restraint/detention<br>- Prompt assessment for patient recommended under Section 9 or apprehended under Section 10 of Mental Health Act.<br>You will<br>- If defusing techniques ineffective, re-triage to category 1 (see above)<br>- Security in attendance until patient sedated if necessary<br>- Intoxication by drugs and alcohol may cause an escalation in behaviour that requires management |

| Triage Code – Treatment Acuity | Description | Typical Presentation | General Management Principles* |
|---|---|---|---|
| 3 – Urgent<br>Within 30 minutes<br>(You would maybe cross the street if you saw this person) | Possible danger to self<br>or others<br>- Moderate behaviour disturbance<br>- Severe distress<br>Australasian Triage Scale states:<br>- Very distressed, risk of self-harm<br>- Acutely psychotic or thought -disordered<br>- Situational crisis, deliberate self-harm<br>- Agitated/ withdrawn | Observed<br>- Agitation/restlessness<br>- Intrusive behaviour<br>- Confused<br>- Ambivalence about treatment<br>- Not likely to wait for treatment<br>Reported<br>- Suicidal ideation – with or without plan<br>- Situational crisis<br>- Self harm, no plan to continue<br>Presence of psychotic symptoms<br>- Hallucinations<br>- Delusions<br>- Paranoid ideas<br>- Thought disordered<br>- Bizarre/agitated behaviour<br>Presence of mood disturbance<br>- Severe symptoms of depression<br>- Withdrawn/uncommunicative and/or anxiety<br>- Elevated or irritable mood(mania) | Supervision<br>Close observation (see definition below)<br>- Do not leave patient in waiting room without support person<br>Action<br>- Alert mental health triage<br>- Ensure safe environment for patient and others<br>Consider<br>- Re-triage / assessment if evidence of increasing behavioural disturbance i.e.<br>  - Restlessness<br>  - Intrusiveness<br>  - Agitation<br>  - Aggressiveness<br>  - Increasing distress<br>- Inform security that patient is in department<br>- Intoxication by drugs and alcohol may cause an escalation in behaviour that requires management |

# Mental Health Triage Tool

| Triage Code – Treatment Acuity | Description | Typical Presentation | General Management Principles* |
|---|---|---|---|
| 4 – Semi-urgent<br>Within 60 minutes<br>(You would not feel the need to cross the street if you saw this person) | Moderate distress<br>Australasian Triage Scale states:<br>- Semi-urgent mental health problem<br>- Under observation and/or no immediate risk to self or others<br>- Medication compliance<br>- Feeling low/ not suicidal | Observed<br>- No agitation/ restlessness<br>- Irritable without aggression<br>- Cooperative<br>- Gives coherent history<br>Reported<br>- Pre-existing mental health disorder<br>- Symptoms of anxiety or deperession without suicidal ideation<br>- Willing to wait | Supervision<br>Intermittent observation (see definition below)<br>Action<br>Discuss with mental health triage nurse<br>Consider<br>- Re-triage if evidence of increasing behavioural disturbance i.e.<br>  - Restlessness<br>  - Intrusiveness<br>  - Agitation<br>  - Aggressiveness<br>  - Increasing distress<br>- Intoxication by drugs and alcohol may cause an escalation in behaviour that requires management |

| Triage Code – Treatment Acuity | Description | Typical Presentation | General Management Principles* |
|---|---|---|---|
| 5 – Non-urgent<br>Within 120 minutes | No danger to self or others<br>- No acute distress<br>- No behavioural disturbance<br>Australasian Triage Scale states:<br>- Known patient with chronic symptoms<br>- Social crisis, clinically well patient | Observed<br>- Cooperative<br>- Communicative and able to engage in developing management plan<br>- Able to discuss concerns<br>- Compliant with instructions<br>Reported<br>- Known patient with chronic psychotic symptoms<br>- Pre-existing non-acute mental health disorder<br>- Known patient with chronic unexplained somatic symptoms<br>- Request for medication<br>- Minor adverse effect of medication<br>- Financial, social, accommodation, or relationship problems | Supervision<br>General observation (see definition below)<br>Action<br>- Discuss with mental health triage<br>- Refer to treating team if case-managed |

Management Definitions

Continuous visual surveillance = person is under direct visual observation at all times
Close observation = regular observation at a maximum of 10 minute intervals
Intermittent observation = regular observation at a maximum of 30 minute intervals
General observation = routine waiting room check at a maximum of 1 hour intervals
* Management principles may differ according to individual health service protocols and facilities.
Australasian College of Emergency Medicine (2000). Guidelines for the implementation of the Australasian Triage Scale (ATS) in Emergency Departments.

# The Mental State Examination

A clinician assesses aspects of a person's mental state across the assessment process, from the initial observation of the person and while taking their history. A detailed Mental State Examination encompasses the full range of a person's expression and not just what they say. The clinician will observe a person's apparent mood, how they behave in the session, note details of their speech and how the person is responding during the interview, and in turn how the clinician is responding to the person. A thorough assessment of mental state will cover the following areas (NB Many of these overlap but have been described separately for clarity).

Appearance and general behaviour: This includes the person's facial expressions, body language, posture, gestures, manner, attire and grooming. These may give clues to the person's personality, mood, awareness of social conventions and ability to take care of themselves. A psychotic person may behave very bizarrely, or a depressed person may appear unkempt or disheveled.

Attitude towards the interviewer: This includes whether the person was suspicious or hostile, evasive, tentative, or co-operative and whether the interviewer felt some rapport had been established. It is also important for the interviewer to consider their reaction to the person. The degree of rapport or therapeutic engagement established is directly related to how willing a person will be to disclose painful or difficult material. This is crucial in assessing for suicide risk as a disengaged or obviously reluctant person may not give a full account of their risk or intent if they do not trust the interviewer or want to be taking part in the assessment process. In such circumstances, it is particularly important to give weight to information from others and to balance this against any denials of suicidal intent during the interview that appear contradictory to these reports.

Affect/mood: Affect refers to the prevailing emotional tone of the interview, and is assessed through the person's facial expression, body posture, and tone of voice. Mood is more of a prolonged feature that is reported by the person (and others if available). A clinician should look for how emotionally responsive a person is during an interview, whether they appear fl at and apathetic or at the other extreme, labile or irritable. It is also important to consider whether their reported mood matches their displayed mood (eg, someone who is speaking slowly, wringing their hands and is intermittently tearful but says they are 'fine' needs to be assessed further). Important dimensions of mood to assess for include depressed mood (eg, sad, tired etc), elated or euphoric mood, irritability and agitation. A clinician should also be aware of obvious signs of tension (eg, sweating, stammering, trembling) and reported anxiety.

Psychomotor activity: This refers to internally driven behaviours, for example, drumming of fingers that may represent feelings of anxiety. Typical behaviours that the clinician should be aware of include restlessness/agitation; slowing of movement, speech and thoughts; questions answered after prolonged delays.

Speech: A great deal of additional information can be gained from listening to how a person speaks and not just the content of their speech. The clinician should listen for the loudness and speed of speech and how effective the person is in getting their point across. For example, a person who has pressured speech may feel compelled to talk and the listener may gain the impression that their ideas flow faster than they can express them. A depressed person may demonstrate 'poverty of speech', such as responding after delays with 'I don't know', or very short or monosyllabic replies.



# The Mental State Examination

Thought content: Inferences about a person's thinking are made on the basis of their report of symptoms. A careful exploration of a person's presenting difficulties and history is crucial if the clinician is to gain an accurate picture of their thinking. In assessing for suicidality a clinician will pay particular attention to suicidal ideation and depressive thoughts (eg, 'I'm no good, I'd be better off dead'). The clinician should ask the person about their beliefs about themselves, others and their place in the world, and screen for the presence of delusional beliefs, anxious thinking, obsessions etc.

Thought process: Again, the clinician makes inferences about a person's thinking based upon their speech. When assessing thought processes the clinician is looking for evidence of the way the person constructs their thoughts, including sequence and speed. Marked difficulties in thought processes may include perseveration (ie, when a person seems to be stuck on one theme), tangentiality (ie, easily side-tracked without being able to return to the initial topic), or loose associations (ie, apparent absence of logical thought processes). A clinician will also be looking at the person's style of thinking (eg, do they worry or ruminate over their misfortunes, are they preoccupied with certain topics/events/ideas? For example, a person may report negative selftalk as above, but then also report that one thought triggers another related one, and another, until they feel like they can't 'shut them off').

Perceptual disturbance: In this instance the clinician is specifically screening for the presence of hallucinations. These may occur across any of the five senses. If the person is suicidal and hearing voices, care must be taken to assess whether these voices are command hallucinations (ie, telling the person to act) and how compelling these commands are. It is also important to assess for the degree of fear associated with the experience of hallucinations as this may also increase a person's suicide risk (ie, ending one's life to escape the feared hallucination).

Orientation: Briefly, this is a screen to check that the person knows where they are, who they are and why they are being interviewed. This series of questions is especially important if the person has come into the emergency department (eg, following an overdose). If the person is disoriented to place, person or time, careful examination is needed to rule out delirium, intoxication or organic complications. Disorientation in time, especially the passage of time (such as the length of the interview), is the most sensitive of these tests of orientation.

Insight and judgment: A clinician needs to judge how much the person is aware of their present circumstances, their health and the reasons for their presentation to mental health services or an emergency department. If a person is assessed as being significantly impaired in their judgment, in a way that places them at risk of harm to themselves or others, they can be compelled to have treatment under the Mental Health (Compulsory Assessment and Treatment) Act 1992.

Intelligence/cognitive function: For the purpose of a suicide assessment a broad estimation of a person's intellectual functioning is sufficient. This can be inferred from the content of their speech, educational history and vocational background. Other aspects of cognitive functioning to screen for are attention and concentration and any memory problems. The clinician can be aided in this by using the Mini Mental State Examination.

Suicide Guideline for PDF.indd (https://www.moh.govt.nz/notebook/nbbooks.nsf/0/318A1334B4C5FAC1CC25730E007D4DFC/$file/Suicide_Guideline.pdf)

# RAPID ASSESSMENT OF PATIENTS IN DISTRESS

These questions are to assist in the assessment of people with potential mental health problems in the emergency department, and to consider urgency of need for mental health referral and security measures.

Name: Address: DOB:
Next of Kin: Telephone: NHI:

***Commentary***

Use Mental State Examination if disoriented.

Consider safety issues for person and staff.

Use prompts over page to assess for suicide.

*** May need to institute measures to prevent inappropriate self discharge prior to full assessment NOW.**

Is the person too sedated, sleepy or disoriented to give an adequate history?

Does the person need an interpreter?

Enquire about old notes/any existing mental health files.

Start a problem list.

Seek consent to contact whānau/ family/friends.

Check if they are from this area.

Do you need to contact anyone immediately or can it wait until the following morning?

Fax contact information.

WHAT IS THE PROBLEM?

Disorientation in:

- ☐ Time?
- ☐ Place?
- ☐ Person?

- ☐ Suicidal ideation*
- ☐ Anxiety/panic attacks
- ☐ Thoughts disordered
- ☐ Bizarre behaviour*
- ☐ Depressed
- ☐ Intoxication*
- ☐ Hallucinating
- ☐ Aggression*
- ☐ 'Stress'
- ☐ Drug and alcohol withdrawal*
- ☐ Delusional
- ☐ Agitation*

WHY NOW?

- ☐ Self-referral
- ☐ Concern from others
- ☐ Personal crisis
- ☐ GP referral
- ☐ Person won't talk
- ☐ Mental Health Act 1992
- ☐ Police presentation
- ☐ Deliberate self-harm

CAN I GET AN ADEQUATE HISTORY NOW? ☐ Yes ☐ No

IS THERE A PSYCHIATRIC HISTORY? ☐ Yes ☐ No

If yes, working diagnosis? _____

Current medications? _____

Legal status? ☐ Mental Health Act 1992 ☐ Informal

Is there an existing management plan? ☐ Yes ☐ No

WHAT DOES THE PERSON WANT?

- ☐ Someone to talk to
- ☐ Accommodation
- ☐ They don't know
- ☐ Psychiatric input
- ☐ Medication
- ☐ Inpatient treatment
- ☐ Detox
- ☐ Outpatient treatment

WHERE CAN I GET INFORMATION FROM? (note contact details)

- ☐ Whānau/family members or friends? ☐ GP?
- ☐ Community health centre? ☐ Voluntary agencies?
- ☐ Case management? ☐ Therapist/counsellor?
- ☐ NGOs? ☐ Psychiatrist? ☐ Psychologist?

IS ANYONE ELSE WITH THEM OR IN THE WAITING ROOM?

Does the person have children? ☐ Yes ☐ No

Where are they now? _____

CAN YOU TELL ME HOW YOU ARE FEELING AT THE MOMENT?

Adapted from Wilhelm, Kotze, Ballard and Hudson, Consultation Liaison Psychiatry and Emergency Departments, St Vincent's Hospital, Sydney. 2001.



These questions will help determine the level of observation needed in the emergency department.

***Commentary***

Consider whether the person is safe to be alone.

Risk factors include:

- definite plan
- hopelessness
- severe depression
- psychotic symptoms
- recent discharge from a psychiatric unit
- use of alcohol, street drugs, particularly recent escalation
- recent suicide attempt
- single men: young, elderly
- homelessness
- medical illness
- history of childhood abuse
- recent suicide attempt by a whānau/family member or a friend.

**IF YOU SUSPECT A PERSON IS CARRYING A WEAPON NOTIFY SECURITY NOW**

Predictors of violence include history of impulsivity and previous violence, alcohol and substance use, antisocial traits/behaviours.

Complete a physical examination.

Make triage decisions and decide whether to refer for a psychiatric assessment.

Does the Mental Health (Compulsory Assessment and Treatment) Act 1992 need to be applied? Is a Duly Authorised Officer required?

Does Child, Youth and Family Services need to be called (to offer 'care and protection' to under 17 year old)? (0508 FAMILY)

Involve social supports in discharge planning.

SUICIDE ASSESSMENT

Have you had thoughts that life isn't worth living? ☐ Yes ☐ No
Have you thought of harming yourself? ☐ Yes ☐ No
Are you thinking of suicide? ☐ Yes ☐ No
Have you tried to harm yourself in the past? ☐ Yes ☐ No
If yes, how many times? _____
When was the most recent time? _____
How often are you having these thoughts? _____
Have you thought about how you would act on these (is there a plan)? *(Does this plan seem feasible? Are the methods available? Is it likely to be lethal?)*

_____

Have you thought about when you might act on this plan?
Are there any things/reasons that stop you from acting on these thoughts? _____
Do you know anyone who has recently tried to harm themselves?

IF A SUICIDE ATTEMPT HAS BEEN MADE

What did you hope would happen as a result of your attempt? *(Did they want to die, or end their pain?)* _____
Do you still have access to the method used? ☐ Yes ☐ No
Did you use alcohol or drugs before the attempt? What did you use?
Do you have easy access to a weapon? ☐ Yes ☐ No

HAVE YOU THOUGHT OF HURTING ANYONE ELSE? ☐ Yes ☐ No
If yes, have you acted on these thoughts? ☐ Yes ☐ No
Have you been involved in any fights recently? ☐ Yes ☐ No
If yes, were you using drugs or alcohol at the time? ☐ Yes ☐ No

DO YOU FEEL SAFE AT THE MOMENT? ☐ Yes ☐ No

WHO DO YOU LIVE WITH?

Is there anyone at home right now? ☐ Yes ☐ No
Is there anyone we need to call? _____

HOW IS YOUR GENERAL HEALTH?

WHAT TRIGGERED THESE THOUGHTS/ACTIONS?

*(What are key stressors, for the person, eg, financial, relationships, employment, bereavement?)*

What lead you to try and take your life? _____
What lead you to having these thoughts? _____
What has been going on in your life lately? Are there any things that have been worrying or upsetting you? _____

WHAT WOULD MAKE A DIFFERENCE AT THE MOMENT?

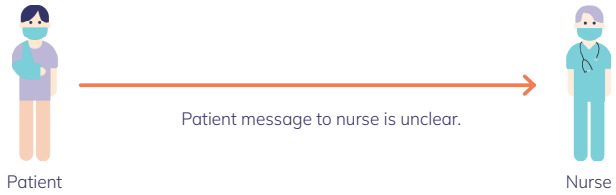
*(Can the person see anything changing in their situation?)*

Can we make a list of the main problems? _____
What can we do now? _____
What needs to be done later? _____

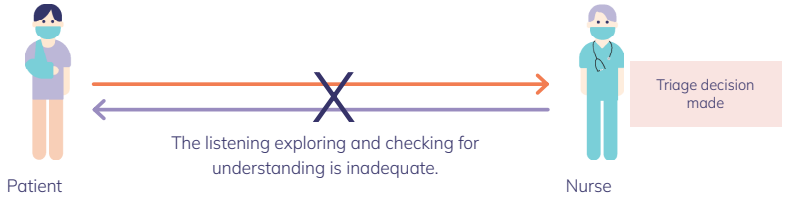
https://www.moh.govt.nz/notebook/nbbooks.nsf/0/318A1334B4C5FAC1CC25730E007D4DFC/$file/Suicide_Guideline.pdf

# Communication Issues at Triage

**Phase 1**




**Phase 2**




**Phase 3**




**Phase 4**




INFLUENCING FACTORS

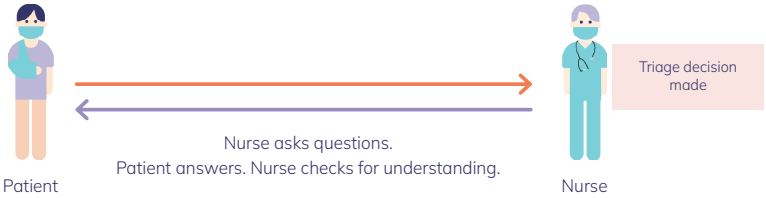


# Communication at Triage

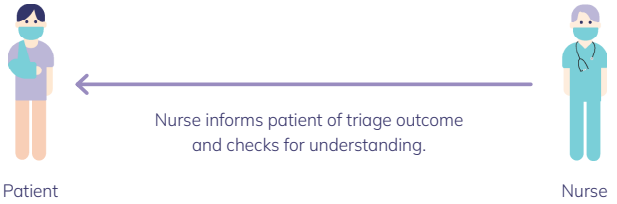
**Phase 1**



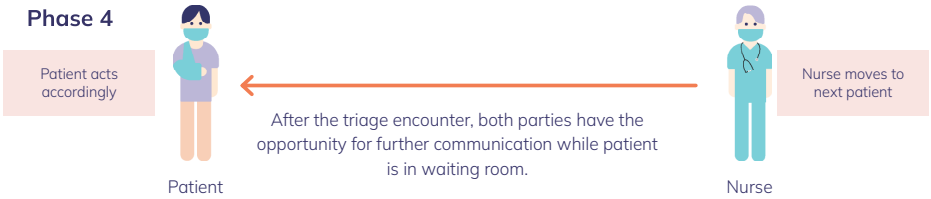

**Phase 2**




**Phase 3**




**Phase 4**




INFLUENCING FACTORS

# Factors Affecting Communication

## Physical environment:

The presence of barriers such as bullet-proof glass, desks, lack of privacy, distracting noise and movement of people throughout the area all impact on the triage communication process. It is often the effort displayed by the Triage Nurse that will overcome these barriers and reassure the patient that their communication with the nurse is private, thorough and confidential.

## Time constraints:

The triage assessment generally should take no more than two to five minutes with a balanced aim of speed and thoroughness being the essence

## Language use:

The use of jargon, be it medical jargon or 'street talk', can result in misinterpretation as profound as that which would occur between two people actually speaking different languages. For example, 'voiding' is a term commonly used in medicine, but may mean something completely different to a layperson, whereas 'doing a wee' would be understood by most people. Consider language barriers for example non-English speaking patients and the use of translators. What considerations would you need to think about when using a translator? What is lost in translation?

## Non-verbal behaviours:

Body language, facial expressions and tone of voice in both the patient and the nurse during the encounter are equally significant aspects of communication.

## Cultural diversity:

These include differences in age, gender, ethnicity, language, religion, socioeconomic status and life experience. For example, an elderly man may be reluctant to discuss some issues with 'a lass who looks younger than his granddaughter', so effort must be put into building a professional rapport. How does the treaty of Waitangi affect Triage?

## Nature of the health concern:

Health concerns that are highly sensitive, embarrassing or anxiety-producing will influence the way in which the information is communicated by both the patient and the Triage Nurse. The avoidance of key terms and the use of euphemisms may lead to distortion of the messages sent and received.

## Expectations and assumptions:

Individuals present to triage with expectations of what will happen. These expectations are influenced by their perception of the urgency of their health concern and by their past health care experiences and may at times be unrealistic. The Triage Nurse's familiarity with the triage environment and with patients, together with the attitudes and behaviors of other Urgent care and emergency staff, can all have both positive and negative influences. Although such influences can aid in early symptom recognition, they can also potentially lead to inappropriate assumptions and bias. Burn out of triage staff and biases towards types of patient's effects communication at triage, how do we keep ourselves from burning out and creating biases?



## Emotions:

Individuals - including both patients and nurses react to stress and anxiety in different ways and with varying intensity. These reactions can impact upon the person's ability to provide coherent information and their ability to answer questions clearly. The Triage Nurse's ability to remain calm and achieve effective communication within this environment is paramount. Tricks and tips to aid in communication when emotions are high will be discussed in face-to-face sessions.

## The Four Basic Human Needs

| Basic human need | Common signals that this need is not being met | Suggested strategies to fulfill this need |
|---|---|---|
| To be understood | – Repeating the same message<br>– Speaking slowly and/or loudly<br>– Getting angry<br>– Bringing a support person to speak for them | – Seperate emotions from patient<br>– Ask questions, shifting the focus from the emotions to exploring the health concern<br>– Acknowledge their feelings, emphasise with their concerns<br>– Reflect your understanding<br>– Inform them of what will happen and why<br>– Do not take expressions of anger personally<br>– Check your own reactions |
| To feel welcome | – Looking around before entering<br>– Looking lost or unsure | – Provide a warm and friendly welcome<br>– Use appropriate language<br>– At the end of the triage encounter keep communication lines open |
| To feel important-one's self-concept | – Drawing attention to themselves<br>– Getting angry<br>– Appearing helpless<br>– Loss of control | – Call the person by their name<br>– Acknowledge their concerns<br>– Tune into their individual needs<br>– Allow anger to diffuse, listen, say nothing<br>– Allow the person to release their emotions |
| Need for comfort--psychological and physical | – Appearing if at ease<br>– Nervouse or unsure<br>– Requesting assistance/help | – Explain the procedures carefully and calmly reassure |

# Rural and Remote Nursing Issues

### Multiple jobs:

Due to the spasmodic nature of the need for triage, rural Triage Nurses often have other jobs as well. Unlike their metropolitan counterparts, rural nurses do not dedicate their time to the triage desk. This can impact upon their opportunities for learning and maintaining skills through consistent practice.

### Lack of a 'safety net':

The Triage Nurse in a rural or remote environment may well be alone in the facility, with no one around to provide support or advice.

### Lack of other options for care:

Patients in this environment cannot easily be triaged elsewhere in the immediate term. When the decision to triage elsewhere is made, consideration must include the distance and the safest way for the patient to travel.

### Time issues:

The initial 'wait' time for patients is often not the key issue in this environment; rather, it may be the time it takes to get them to the hospital, and, once they have arrived, how to get them the most appropriate care in the fastest possible time.

### Delivery of initial emergency care:

This can be a source of anxiety for staff. Dealing with the unexpected, with limited support or specialized back-up, means that the rural Triage Nurse needs a broad range of knowledge and skills.

### Personal and departmental safety:

This can be a potential problem. Triaging without security, often without even another nurse, or a local police station is a major source of anxiety in rural and remote triage practice.

### The triage process may occur outside the hospital setting:

For example, the triage process may occur as part of a district nursing community health care role, or via telephone, as patients try to avoid the inconvenience of traveling long distances to access health care advice or treatment. However, it must be emphasized here that the ATS is a face-to-face tool, and local protocols must guide other triage types.

### Lack of anonymity within the community:

This may result in a nurse being contacted at home, or within a social setting, to perform a triage assessment, and can present issues related to confidentiality. It may result in a nurse caring for a friend, acquaintance or relative. Personal relationships can also be unwittingly abused by patients seeking special treatment, which may make triage decisions more difficult to make.

### Decisions may carry enormous financial or social ramifications for patients and their families:

This may be the case especially when a decision is made to triage away from the local health facility.

### Decisions may carry collegial ramifications:

This may be the case especially if a decision concerns the local doctor in a one-GP town, as a working relationship must still be maintained.

### Awareness of budgetary pressures:

Within a small facility this can be heightened, due to there being fewer buffers between hospital administration and the nurse.

### Knowledge of the community:

It can be easy to make assumptions when you know the community and the individual community members well. The triage nurse needs to be very careful to obtain detailed and clear information concerning the patients that they are triaging and the history of their current complaint.



# DNW Policy Triage

## Background and statement

Person-centered care promotes recognition, understanding, and consideration of each person's unique needs and circumstances, including culture, beliefs, values, traditions, family situations, social circumstances, lifestyle, and preferences. This approach supports and encourages each person to fully participate in decision-making as much as possible or desirable in their own care which may mean they choose to leave without care.

However, Doctors, Registered Nurses and Paramedics retain a duty of care for all patients and the follow-up of patients that DNW should be adhered to in accordance with this policy. To reduce the risk of adverse outcomes. Especially in a patient with high risks and /or poor health literacy or with a question around competence.

## Documentation

If a patient is not present in the waiting room when called three times, this is to be documented on the DNW form below at the time of calling the patient.

| DNW | Responsible clinician | Date/ Time | Signature | Action |
|---|---|---|---|---|
| Patient not found/ Present after three calls | Caller | | | |
| Charge Nurse or associate charge nurse notified | CNM or ACNM signs | | | |
| Doctor or Medical director notified if applicable | Doctor or Medical Director signs | | | |

## Process

The patient is called three times at least 10 minutes apart. If not found, all three DNW episodes are to be documented in the patient management system (PMS). Following the third patient call, enter the DNW details. The staff member who called the patient notifies the triage nurse/charge nurse manager or associate charge nurse manager, who then ensures the DNW is entered into the appropriate patient database and is to request the Patient's Medical Record for review (if available).

The most senior RN (as discussed above) is to then review the information recorded at triage and subsequently the nature of the patient's documented presenting complaint and is to check the Patient Management System (PMS) for any history of DNW.

**The most senior RN is to consider the risk to the patient of DNW with particular consideration of:**

1. the ATS triage score and assessment including HR, Sp02, RR and pulse characteristics

2. whether the patient is paediatric (in general under 18 years)

3. children identified as having or requiring a report of concern

4. Whether the patient has presented previously but DNW or did not complete their assessment on one or more prior presentations

6. whether the patient is in a high-risk demographic / vulnerable population

7. Any patient presenting with mental health concerns or has a known mental health history (confirmed via the medical record) is to be considered, at a minimum, a moderate risk under the definition of this policy

8. Any patient presenting with deliberate self-harm, suicidal ideation or thoughts to harm others are to be considered high risk under the definitions of this policy.

Once the senior RN has assessed the patients risk as a result of their DNW status, they are to discuss the risk assessment with the relevant staff for escalation purposes for medium and high risk patients

## Risk assessment:

When it comes to DNW Patient Risk Assessment It is acknowledged that there is no perfect predictor for knowing what low, moderate and high risk is. Therefore, risk assessment will rely on your best clinical assessment each time. Low risk has been defined as requiring no action, moderate risk requires deferred action and high risk requires immediate action.

**Low Risk** – No Action where the senior RN determines that the risk to the patient is low, no further action needs to be taken and this decision and supporting evidence is to be entered into the form above and signed.

**Moderate Risk** – Deferred action where the senior RN or doctor determine a moderate risk to the patient who did not wait, the patient is to be referred for active follow up by way of phone call during the next available business hours. This follow up may be by the senior RN or through handover to another senior RN during a normal business hours shift. Where the patient is engaged with services, social work, or other appropriate persons, contact can be attempted through these means. In these cases, active efforts are to be undertaken to make phone contact with the patient and encourage them to return for assessment and if not to contact their GP and the conversation is to be documented in the medical record. Inability to contact the patient is to be documented and further risk assessment made by the senior RN in collaboration with a doctor.

**High Risk** – Immediate action where the senior RN and the doctor determine that there is a high immediate risk to the patient who did not wait to be seen. The senior RN is to immediately attempt contact with the patient in an effort to encourage them to return. The mental health services or other appropriate support can be enlisted if appropriate and available, especially for vulnerable patients such as elderly patients and/or paediatric patients. Where these attempts at contact are unsuccessful but the patient is still considered to be at significant risk, request for assistance to locate the patient is to be escalated appropriately to police. All actions and conversations are documented in the patient's medical record.



## Special considerations

Special consideration for elderly, paediatrics, mental health patients and vulnerable patients, person-centred care should focus on the social, emotional and cultural needs of the person and ensure patients and families/carers are involved in patient's care. Person centred care should recognise the diversity of people and communities including across gender, age, language, geographic location, sexual orientation, religious beliefs, family responsibilities, life experiences and educational levels.

## Definitions

**Did not wait (DNW)** Patients who present to Clinics / Services and did not wait to be treated after triage are classified as 'did not wait' for reporting purposes.

**Senior (RN)** Most senior nurse on shift.

## Records Management

All medical records/ notes must be stored in the approved PMS and Records Management System in accordance with PMS Policy. All clinical records must be managed in accordance with patient privacy and confidentiality.

The Senior Doctor or senior RN are to review DNW statistics and follow up outcomes weekly. To monitor for DNW trends, raising any unwanted clinical variation with those with operational accountability and supporting formulation and documentation of any clinical risks or improvement plans. Review of this procedure is to be carried out every three to five years from implementation of the procedure.

## References:

1. .Australian Government Department of Health and Ageing. (n.d.). Triage Education Kit. Retrieved September 2022, from Australian College for Emergency Medicine: https://acem.org.au/getmedia/c9ba86b7-c2ba-4701-9b4f-86a12ab91152/Triage-Education-Kit.aspx

2. Committee, N. E. (Amended 2015, Amended March). Mental Health Ed Guide. Retrieved from NSW Health: https://www.health.nsw.gov.au/mentalhealth/resources/Publications/mental-health-ed-guide.pdf

**This course is based on the Australasian Triage Scale and The Standard 2015. This content has been adapted to meet the industry needs of Urgent Care and PHC in New Zealand**

# WORKSHOP PART A

Questions
Discussion Points
Group Activities
Case Studies





## WORKSHOP PART - A

Question 1

The primary aim of an acute ATS triage system is to:

A. Define the Acute/ Urgent patient case-mix

B. Ensure that Acute and Urgent care is delivered according to time of arrival

C. Ensure that Acute and Urgent care is delivered according to clinical need

D. Prevent Acute and Urgent clinic overcrowding

Question 2

In an Acute and Urgent triage system, 'urgency' refers to:

A. The complexity of the patient's condition at a particular point in time

B. The clinical features of the patient's condition

C. Severity

D. Complexity

## Question 3

When talking about the 'reliability' of a triage scale, we are referring to:

A. Its utility (i.e. how easy the scale is to use)

B. How consistently the scale is applied to the same cases by different users

C. How sensitive the scale is to capturing novel presentations of high acuity and its ability to measure what it sets out to measure

D. Correlation with diagnosis

## Question 4

In terms of precision, five-tier triage scales are:

A. Equal to three-tier scales

B. More reliable than three or four-tier scales

C. Equal to four-tier scales

D. Both (a) and (c) above.

## Discussion Point

What is 'under-triage' and 'over-triage'?

## Discussion Point

What factors do you think are likely to contribute to each?

## Discussion Point

How might these factors be minimized in actual practice?

## Question 5

The term 'time-to-treatment' in the ATS refers to:

A. The average time that a patient in that category should wait for assessment and treatment

B. The maximum ideal time that a patient in that category should wait for

C. Assessment and treatment

D. The minimum time that a patient in that category should wait for assessment and treatment

E. The performance-indicator threshold for that category

## Question 6

The performance-indicator threshold for the ATS categories are:

A. The minimum percentage of presentations per ATS category that are expected to achieve the time-to-treatment objective

B. The maximum percentage of presentations per ATS category that are expected to achieve the time-to-treatment objective

C. The degree to which clinicians agree on the allocation of a triage code across a population

D. The minimum time that an individual patient in that category should wait for assessment and treatment

## Question 7

Commonly referred to as 'footprints', the distribution of presentations across the five categories of the ATS can be used to:

A. Determine the accuracy of individual triage decisions

B. Determine whether nurses are systematically under or over-triaging certain groups of patients

C. Benchmark triage categories within Clinics with similar demographic profiles

D. Both b) and c) above

## Question 8

Environmental factors such as staffing, skill mix, and clinics acuity level:

A. Are considered valid reasons for upgrading a triage code

B. Must not influence urgency allocation

C. Can lead to under or over-triaging

D. Both b) and c) above

## Discussion Point

1. How could you assess the consistency of triage within your own clinic?

## Discussion Point

What strategies, if any, might be employed to improve triage consistency within your own clinic?

# Communication Issues at Triage

## Learning Activity 1

Consider this diagram 'Communication issues at triage' (see page 28).

What factors might cause these difficulties at each phase?

Discuss the impact(s) that the identified communication problems at each phase may have on the assessment of urgency and on the use of the ATS. Discuss with the group.

## Learning Activity 2

In the diagrams, Communication Issues at Triage on page 28 and Communication at Triage on page 29, there is a comment in Phase 4 that states 'Patient acts accordingly'.

Compare and contrast the communication process in the two diagrams. Discuss the differing patient behavior that you predict may be evidenced with each process. Discuss with the group.

## Learning Activity 3

Refer to page 30 Factors Affecting Communication, and create a list of influencing factors that are potential barriers to communication within your workplace.

Identify realistic measures that can be taken to overcome these barriers.

## Learning Activity 4

Please refer to page 31 The Four Basic Human Needs. An elderly woman well known to the Clinic is brought into triage via POAC in an ambulance. The ambulance officer states 'We've brought in Mavis again. She's complaining about the usual'. What factors do you need to be aware of to ensure that your assessment of Mavis is complete?

Discuss with the group.

## Learning Activity 5

Please refer to page 31 The Four Basic Human Needs. Two men present with central chest pain. One is obviously terrified and pleading for help, while the other is loudly and angrily demanding immediate attention. Discuss the different strategies you would use to extract indicators of urgency from each man. Identify and discuss your reactions to the different behaviors and how these reactions may influence your ability to assess the urgency.

## Learning Activity 6

Recall a communication situation at triage that you found challenging. Describe the event. What emotions were being expressed? What aspects did you find most challenging and why? What was your reaction? How did the patient's emotions and your reaction impact the assessment process?

Now, using the same communication situation, refer to the table on page 31. Which of these four basic human needs was potentially being expressed by the patient?

On reflection, what strategies could have been implemented to address these potential needs? Discuss with the group.

## Learning Activity 7

As a group, discuss what communication strategies might be effective when dealing with each of the following patients at triage:

- an intoxicated person
- a verbally abusive person
- a confused elderly person
- a prisoner in police custody
- a two-year-old child
- an intellectually disabled person
- a person who does not speak English
- a young woman who has been sexually abused
- a hysterical mother on the telephone

# Letters to the Editor

## Thumbs Down to Local After-Hours

*I took my three-year-old to the Urgent Care Clinic last Saturday evening and was not impressed. My son had suffered a major head injury after falling from his bike, and I was told by the girl at the desk that I would have to wait at least two hours to see the doctor.*

*Thankfully, I was able to contact my local GP who arranged to see and treat my son immediately and we were back home within half an hour.*

*I am surprised that critically ill children are not considered a priority by the Clinic.*

*Signed,*
*Disgusted.*



Learning Activity 8

A. What does this letter tell you about the triage encounter?

B. What do you think happened?

C. Identify some strategies a Triage Nurse could implement to avoid this type of scenario?

## *Case Study 1*

Reese, 31 years, suffers from migraines. Today she came to the Clinic with her sister. She has had an eight-hour history of global headache, vomiting and visual disturbance. She has taken her usual medication (imigran). But says it is 'not working'. Her heart rate is 96 beats per minute, respiratory rate 28 breaths per minute. She is afebrile and rates her pain seven out of ten.

## Case Study 2

Maree is a 32-year-old woman who presents complaining of ‘palpitations’. She is 30 weeks pregnant (G3PI) and is normally well. She was doing the vacuuming when her palpitations started, she complains of mild chest pain that is dull in nature and a mild shortness of breath. Her respiratory rate is 24 breaths per minute; SPO, is 98 percent; and heart rate is 162 beats per minute, her skin is pale, cool and dry; pulse is weak and thready; and Glasgow come score is 15 out of 15.

## Case Study 3

Elliot is 27 years old. He injured his back yesterday lifting a heavy box at work. He had been managing the pain at home, however today it is ‘much worse’. He was unable to get an appointment with his local doctor, so he has come to the clinic. He rates his pain ‘Five out of ten’ and has taken two Panadeine Porte and two Nurofen tablets in the past ½ hour.

# WORKSHOP PART B

Questions
Discussion Points
Group Activities
Case Studies



# WORKSHOP PART - B

## Discussion Point 1

Consider the following question and discuss your answers with the group.

1. Please refer to information on pages 4 and 6, what are the elements of the primary survey and how can you assess them at triage?

## Discussion Point 2

Please refer to the information on page 6, for each of the following physiological abnormalities, discuss their significance, how they would change your triage decision and the allocation of an ATS category.

**a) Tachypnoea**

i. respiratory rate 22/minute

ii. respiratory rate 28/minute

iii. respiratory rate 36/minute

**b) Tachycardia**

i. respiratory rate 106/minute

ii. respiratory rate 128/minute

iii. respiratory rate 152/minute

**c) Hypotension**

i. systolic blood pressure 90 mmHg (a present radial pulse)

ii. systolic blood pressure 70 mmHg (an absent radial pulse)

## Patient Scenario 1

A 32-year-old male presents with two hours of increasing shortness of breath. He has audible stridor and is unable to speak. He has a respiratory rate of 36 breaths per minute, maximal use of accessory muscles and oxygen saturation of 92 per cent despite supplemental oxygen at 10 litres per minute. His heart rate is 132 and his skin is pale, cool and moist.

What key features of this patient's presentation would make him an ATS category 1?

## Patient Scenario 2

A 22-year-old male presents with a painful, deformed left shoulder. The patient states that he was helping friends to move house when a bookcase fell on him. He has a respiratory rate of 26 breaths per minute and no use of accessory muscles. His heart rate is 118 and his skin is pale, cool and moist.

What key features of this patient's presentation would make him an ATS category 2?

## Patient Scenario 3

A 68-year-old female presents with three hours of increasing abdominal pain. She has a respiratory rate of 24 breaths per minute and oxygen saturation of 93 percent on room air. Her heart rate is 108 per minute and her skin is pale, warm, and dry.

What key features of this patient's presentation would make her an ATS category 3?

## Patient Scenario 4

A 28-year-old female presents with two days of vomiting, diarrhea, and lower abdominal pain. She has a respiratory rate of 18 breaths per minute and a heart rate of 94 per minute. Her skin is pale, cool, and dry and her mucous membranes are moist. She states that her pain is 3/10.

What features of this patient's presentation would make her an ATS category 4?

## Patient Scenario 5

A 40-year-old male presents with a minor laceration to his right forearm. The patient states that he was renovating his kitchen and cutting his hand on a piece of wire. The laceration is one centimeter in length, is well approximated, and is not bleeding. He has a respiratory rate of 16 breaths per minute and his heart rate is 76.

What features of this patient's presentation would make him an ATS category 5?



## Patient Scenario 6

A 20-year-old male presents with a history of asthma and complains of feeling short of breath. What key features of this patient's presentation would make him:

a) An ATS category 1?

b) An ATS category 2?

c) An ATS category 3?

## Patient Scenario 7

A 70-year-old male presents with left loin pain radiating into the groin. What key features of this patient's presentation would make him:

a) An ATS category 1?

b) An ATS category 2?

c) An ATS category 3?

d) An ATS category 4?

e) An ATS category 5?

## Discussion Point 3

Reflect on the mental health presentations you have encountered in your clinical practice..

a) Referring to the information on pages 19 - 27, what communication techniques did you use to gather your assessment data?

b) What cues did you use to identify that the patient had a primary mental health problem?

## Discussion Point 4

a) Referring to the information on pages 19 - 27, identify any difficulties you may have had in the identification and assessment of mental illness at triage.

b) On reflection, would using any of the strategies identified have assisted you in making the triage decision?

## Discussion Point 5

Referring to the information on pages 19 - 27, a 40-year woman, Betty, calls an ambulance and is transported to your Clinic. She reports feeling suicidal. She has tablets and has written a suicide note to her daughter, but she changed her mind and wants help. When she arrives at triage the most notable feature is that Betty is quite inebriated and loud. She wants to be in hospital where she can 'get help and feel safe'. She reports that she 'cannot cope with her family'. She admits to drinking 'a bit too much lately'. She also gives quite a few symptoms suggestive of recent anxiety and depression. She then begins to get very agitated and says she will kill herself if she can't get help.

a) What clinical factors would make Betty:

i. An ATS category 1?

ii. An ATS category 2?

iii. An ATS category 3?

iv. An ATS category 4?

v. An ATS category 5?

b) For each of the above categories, what general management principles would guide practice? For example, supervision requirements, communication techniques and referrals.

## Discussion Point 6

Referring to the information on pages 19 - 27, a well-known aged 52, presents to your Clinic because he is afraid he will act on his suicidal thoughts. He has been married for 32 years, and his wife left him 10 days ago. The local community believes that she is visiting her ill mother in the city. He has managed to function effectively until recently. Two day ago, he had to destroy his stock due to drought conditions. That morning he visited his daughter and grandchild to say goodbye, and they became concerned and brought him to the center. He has access to guns.

What immediate action would need to be taken if you assigned him to:

i. An ATS category 1? _____

ii. An ATS category 2? _____

iii. An ATS category 3? _____

iv. An ATS category 4? _____

v. An ATS category 5? _____



## Discussion Point 7

Referring to the information on pages 19 - 27, an elderly woman is brought to the Urgent Care Clinic by a concerned neighbor. She had been found wandering in a soiled nightdress, wearing high-heeled shoes and a string of pearls. She is odorous, with cracked lips and a rosy complexion. On questioning, she is unable to recognize the neighbor (whom she had known for 16 years) and insists that she was at the hairdressers for her weekly appointment.

What clinical factors would make this person:

i. An ATS category 1? _____

ii. An ATS category 2? _____

iii. An ATS category 3? _____

iv. An ATS category 4? _____

v. An ATS category 5? _____

## Discussion Point 8

A 72-year-old man is brought to the clinic complaining of feeling weak. His wife has recently died. He has a history of COPD and hypertension. At triage, he is distractible and restless.

a) Discuss your triage assessment of this patient. What additional information would facilitate an informed triage decision?

_____

_____

b) After 30 minutes he begins to shout and remove his clothes. What do you do next?

_____

_____

_____

## Activity 1

Relate the issues raised in the rural and remote nursing issues section in the workbook, on page 32. Consider whether you have had any of these issues impact on clinical decision-making in the triage role.

Reflect on what happened in that situation. In what way was your decision influenced and what was the outcome? Were any lessons learned?

## Activity 2

Using the headings from the 'Rural and remote nursing issues' in your workbook, on page 32, identify the issues, differences and challenges related to your own triage context, and list the skills required of the nurse to manage these on a daily basis.

## Activity 3

Discuss the following scenario.

A man presents in the clinic complaining of chest pain and collapses while you are taking his history. There is no Doctor available. Discuss his ATS classification and the differences in the role of the Triage Nurse from this point between a rural facility and a busy urban clinic.



## Activity 4

Nurses identify various benefits and drawbacks in rural practice. Referring to the the information on page 32, consider the issues presented in module six and discuss the potential benefits that rural nursing may offer to your nursing practice.

## Activity 5

Imagine you are a Rural and Remote UCC Nurse, reflect upon your role in the rural facility as the expert generalist.

List the true range of roles and responsibilities that you fill.

Reflect on how prepared, educationally and clinically, you are to fulfil this role.

What resources can you identify to further support your practice?

## Activity 6

Discuss the following scenario.

It is a Sunday night at a rural clinic, after an extremely busy weekend, and your general practitioner has been present almost around the clock. The staff is aware that he is exhausted and he has stated that he only wants to be called if 'it's really an emergency!'

Discuss as a group, the impact that this information puts on the Triage Nurse and the potential influence it may have on the consistent allocation of urgency using the ATS.

## Activity 7

Consider the following scenario.

The mother of one of your children's school friends presents to the triage desk. You know that she has five children and that her husband is working away from the town. She says to you, 'I just want to nip in and ask the doctor a quick question. I won't be long as I've got to go and meet the school bus. Can you please squeeze me in first? Thanks so much!'

Discuss as a group how this situation could be handled.

## Activity 8

Create two lists highlighting the differences between triage in an urban setting and triage in a rural clinic. The lists may include physical, geographical, individual, and professional issues.

| Urban Triage | Rural Triage |
| --- | --- |
| | |
| | |
| | |
| | |
| | |
| | |

## Activity 9

Imagine that a student nurse approaches you and tells you that after she graduates, she wants to work in a small rural facility. She is seeking your advice as to what skills or further studies she needs to develop to prepare her for this role.

What is your advice to her?

## Case Study 1

Rosemary is a 40-year-old woman who presents to triage complaining of abdominal pain. She is 36 weeks pregnant (G5P3) and is normally well. She tells you she has had pain 'on and off' for one week but it has become more severe in the last day. It is difficult for the patient to be precise about the location, but the pain seems to be in the right upper quadrant. She says the pain is 'worse after eating' and today she has vomited twice. She says this pain 'does not feel like labour pains'. Her respiratory rate is 22 breaths per minute, SpO2 is 98 per cent and heart rate is 106 beats per minute. Her skin is pale, warm and dry, and her Glasgow Coma Score is 15 out of 15. Temperature is measured at 37.8°C.

## Case Study 2

Martin presents to the triage desk on his own. He tells you he has pain in his left shoulder after he fell in a driveway. On examination you note that his left shoulder is very swollen. He has very limited range of movement, in fact he can not move the shoulder joint itself, and he rates his pain as 'seven out of ten'. His left radial pulse is present, but he has some 'numbness' around the shoulder area. His arm is in a sling and he smells of alcohol. He says the occurred 'a couple of hours ago'.

## Case Study 3

Cassandra, 15 years, was riding her horse in the bush some 60 km away from town when the animal was startled and threw her about three metres. She was wearing a helmet but it broke in half when her head struck a tree. Her companions noted an initial loss of consciousness, after which she was drowsy and vomiting, but she did not appear to have any injuries elsewhere and she said she had no neck pain when asked. Cassandra was transferred to your UCC in the back of a utility. On arrival she has a Glasgow Coma Score of 8 out of 15.

# WORKSHOP PART C

Questions
Discussion Points
Group Activities
Case Studies



## WORKSHOP PART - C

### Discussion Point 1

Referring to the information on pages 9, 10, 15 and 16, what strategies are currently used at triage to assess pain in your clinic?

### Discussion Point 2

Identify physiological, behavioral, and descriptive indicators of pain, and relate each of these to triage categories.

Discuss the implications for consistency of triage classification that may arise.

## Discussion Point 3

3. Reflect on the following statements related to pain, pain assessment and pain management:

- Analgesia will conceal underlying pathology.
- Patients who receive narcotic medications are at risk of addiction/dependency.
- Treatment with opioids will mask deteriorating neurological function.
- Patients fear that they will be labelled a 'problem patient', and this may affect their self-reporting of pain.
- People with chronic pain become 'used' to their pain, and hence have a higher pain tolerance.
- People with chronic pain are reporting severe pain due to psychological issues rather than the severity of pain itself.

**(a) What does the best available evidence tell us about these myths and misconceptions?**

**(b) How may these myths and misconceptions about pain influence triage decision-making?**



## Question 1

Please refer to the information on pages 11 - 16 for the pediatric questions.

The primary aim of pediatric triage decision-making is to ensure that:

(a) Care is delivered according to clinical need

(b) Children do not wait in the waiting room

(c) Children are seen more urgently than adults

(d) Children do not deteriorate

## Question 2

A physiological approach to triage assessment can be used to evaluate children presenting to the clinic because:

(a) The priorities of assessment and care are the same regardless of age

(b) Children do not wait in the waiting room

(c) This approach does not rely on the evaluation of vital signs which differ vastly with age

(d) Children do not deteriorate.

(e) The differences between adults and children can be accounted for in the assessment and interpretation of findings using this approach

## Question 3

A mother presents to your clinic with a three-month-old baby with fever and poor feeding. Your first priority of assessment is: :

(a) Evaluation of the infant's hydration status

(b) Assessment of airway patency

(c) Establishing the color of the child's skin

(d) Measuring the child's temperature.

## Question 4

The following sign is negatively predictive for serious illness:

(a) Feeding within normal limits

(b) Smiling

(c) Fever

(d) Alterations in activity or conscious state.

## Question 5

A two-year-old child with soft stridor, mild increase in work of breathing, pink and warm skin who is playful during assessment should be allocated the following urgency category:

(a) ATS 1

(b) ATS 2

(c) ATS 3

(d) ATS 4

## Discussion Point 4

What (national or local) policies exist to influence pediatric triage practice, and why have they been implemented?

## Discussion Point 5

How do these policies impact on other patients presenting to the clinic and on the effectiveness of the triage system?

## Question 6

Please refer to the information on pages 8 and 9 to answer the following.

The normal blood pressure changes that occur during pregnancy, develop:

(a) At about 20 weeks gestation

(b) From about 6-8 weeks gestation

(c) During the third trimester

(d) During the second trimester.

## Question 7

A 26-year-old woman in her third trimester presents to the Clinic with a severe headache. What are the primary pieces of information you need to make a triage decision?

(a) Past medical history, presenting history

(b) Conscious state and blood pressure

(c) What medications the woman has taken

(d) Her gestation and presence of fetal movements

## Short Answer Question 1

A 42-year-old woman presents with no foetal movements. Discuss the information you need to determine a triage category.

**Discuss with the group.**

## Short Answer Question 2

A 32-year-old woman presents with PV bleeding. She says that it is possible that she is pregnant, though she's not sure. Describe how you would assess this woman. category.

**Discuss with the group.**

## Short Answer Question 3

A 39-year-old woman who is 27 weeks pregnant presents to the Clinic with chest pain. What are the potential causes of this symptom, and how may they be distinguished?

**Discuss with the group.**

Discussion Point

What are the key questions you would ask about the pregnancy when a pregnant woman presented to triage?

Discussion Point

Acutely unwell pregnant women may remain hemodynamically stable until a sudden deterioration in condition takes place. Why may there be a delay in changes to vital signs in pregnant women?

## Case Study 1

Mohammed is a 24-year-old Somali man who is brought to the UCC by police. He is crying and lying on the floor, rocking. He smells of alcohol, and police say he is a refugee who has recently been released from the asylum seekers detention center in Mount Eden. He has committed no crime, but was apprehended 'directing traffic' in the middle of a busy city highway.

## Case Study 2

Bianca is 24 years old. She has a history of a perianal abscess, which underwent drainage two days ago. She continues to have pain ('six out of ten') and was unable to get an appointment to be seen by her doctor today. She has taken Panadeine Forte with no relief and is also on oral antibiotics.

## Case Study 3

Remo is a 43-year-old male who presents with a two-week history of right renal stones. He now has pain, which he describes as 'colicky' in nature. He rates the pain as 'four out of ten'. He has had no pain relief today.

# WORKSHOP PART D

Questions
Discussion Points
Group Activities
Case Studies



## WORKSHOP PART - D

### Discussion Point 1

Referring to the information on pages 33-35, identify what the identify what the requirements are of the health care facility for the documentation of patients who choose not to wait for treatment or who leave at their own risk.

### Discussion Point 2

Identify what tools and equiptment you will need to conduct triage in the triage area, including any extra resources such as protocols that may be needed.

### Discussion Point 3

Identify the policies and protocols for your health care facility or healthcare sector eg Urgent Care, General Practice, or Rural General Practice that demonstrates the standard of care.

## Patient Scenario

John Oliver was a 25-year-old man who was a regular nightclubber and had spent the night before out with ffriends until the early hours of the morning. Later that day he attended the local clinic and was complaining of a headache. He was accompanied by a friend who waited with him in the waiting room. He was assessed as an ATS category 4 and was seated in the waiting room accompanied by his friend. After two hours, the friend spoke to the Triage Nurse about his concerns for his friend, but John made the decision not to wait to be seen by a doctor and went home. The next day he was found to have died at home.

**(a) What is the role and responsibility of the Triage Nurse to stop John from leaving the clinic?**

**(b) What are the documentation requirements of any interactions with the patient or his friend?**

**(c) What is the procedure for documentation of people who do not wait for treatment at your clinic?**



## Patient Scenario

A five-year-old girl is brought in by UBER with moderately severe asthma. She is accompanied by her 14-year-old brother who is translating for her. Her mother is at home, being unable to attend as she is 38 weeks pregnant and is looking after a three-year-old and a seven-year-old.

What issues does this presentation illustrate? Why?

## Patient Scenario

A 15-year-old girl presents to triage in the presence of police, smelling of alcohol, stating that she was raped in the toilets of the local club.

Outline your responsibilities as a Triage Nurse and describe local practices to manage this case.

## Patient Scenario

A six-year-old presents with his mother with a deep laceration to his arm. His mother states that he fell over in the park. The wound is not actively bleeding, and you assign him an ATS category 4. You place him in the waiting room and 30 minutes later they are gone. You phone the mobile number they have given and find that it is disconnected.

What do you do next?

## Discussion Point

Information about a patient is requested of the Triage Nurse by:

- A police officer
- A child protection worker
- The media
- The patient's employer
- The driver of the other car involved in the accident
- A concerned bystander or witness to the accident
- The patient's parent
- A relative of the patient
- Another member of the Urgent Care Clinic staff.

**(a) What are your responsibilities and how would you deal with this situation ??**

## Discussion Point

Referring to the information on pages 33-35, identify the risks associated with the note 'DNW' (did not wait) being the only documentation by a Triage Nurse of a patient who did not wait to be seen for medical treatment.

# Case Study 1

Lee is a 20-year-old female who presents to the UCC with her mother. Her mother reports that she has had paranoid hallucinations and that since yesterday she has not taken any fluids. She states that her reason for not drinking is that she believes that there are 'spiders and poison around'.

# Case Study 2

Ann is a 16-year-old female who walks to triage with her mother. She reports that she injured her left wrist while playing volley ball. On examination you note good range of movement but she still has some pain. She says the pain is 'three out of ten'.

# *Case Study 3*

Adit is a 15-month-old boy with a two-hour onset of fever and breathing difficulty. He is brought in by his father with an audible stridor at rest.